

510(k) SUBMISSION INTERACTIVE TEMPLATES

Practitioner Reference Forms — Fill, Save, and Reuse in Any PDF Reader

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HOW TO USE: Open in Adobe Acrobat, Foxit, or any PDF reader with form support. Click checkboxes to mark items complete. Click dropdown arrows to select from options. Click text fields to type your device details. Save the completed form as part of your 510(k) submission Technical File or job search portfolio.

All regulatory citations sourced from public FDA records. This document does not represent the views of any regulatory agency or device manufacturer.

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Template I — Predicate Selection Decision Checklist

Use this checklist to evaluate and document each candidate predicate device before finalizing your 510(k) submission strategy. Work through each phase in sequence. A weak predicate is the most common source of FDA additional information (AI) requests.

Subject Device Name:

Candidate Predicate Device:

Subject K-Number (if known):

Predicate K-Number:

PHASE 1 — Initial Eligibility Screen

- ☐ Candidate device was legally marketed before May 28, 1976 (pre-amendments), OR has received FDA 510(k) clearance, OR received a De Novo authorization
→ Pre-amendments devices must not have been subsequently reclassified as Class III. De Novo product codes are valid predicate bases.
- ☐ Candidate device is in the same device category — same FDA product code, or same classification regulation (21 CFR Part and Section)
→ A different product code is not an automatic disqualifier, but requires a stronger intended use equivalence argument.
- ☐ Candidate device is currently legally marketed — verify not withdrawn, recalled, or subject to a prohibition order
→ Check both the FDA 510(k) database and the device recall database (MAUDE) before finalizing predicate selection.
- ☐ Candidate device is NOT a PMA-cleared device
→ PMA-cleared devices cannot serve as 510(k) predicates. Only 510(k)-cleared or pre-amendments devices qualify.
- ☐ Candidate device has not been subject to a Class III determination by FDA that would make it an invalid predicate
→ Check for any subsequent reclassification orders or notices published in the Federal Register.

PHASE 2 — Intended Use Equivalence

- ☐ The predicate's Indications for Use (IFU) statement matches your device's intended patient population (age, disease state, clinical context)
→ Compare patient populations: all must align, or your device's indication must be narrower than the predicate.
- ☐ The predicate's intended body part or anatomical site is the same as your device
→ A cardiac catheter cannot predicate a vascular catheter without a strong intended-use equivalence argument per 21 CFR §807.100(b)(1).
- ☐ The predicate's clinical purpose (diagnostic, therapeutic, monitoring, surgical) matches your device
→ Purpose mismatch is a fatal flaw. FDA's 21 CFR §807.100(b) requires same intended use or no new safety questions from a different use.
- ☐ The predicate's clinical setting (hospital, outpatient, home-use) is consistent with your device
→ Home-use devices face different IEC 62366 usability requirements than professional-use devices — different settings may not support SE.
- ☐ The predicate's contraindications and warnings are consistent with what your device will carry
→ If your device introduces new contraindications not in the predicate, this weakens the SE argument and may require additional evidence.
- ☐ Intended use conclusion is documented: SAME or DIFFERENT — with written justification for any difference
→ FDA requires an explicit statement per 21 CFR §807.100(b)(1). "Different" is permissible if you establish no new safety questions arise.

PHASE 3 — Technological Characteristics

- ☐ Every technological difference between your device and the predicate has been identified and documented
→ Document materials, design features, energy type, software, operating principles — exhaustive comparison required.
- ☐ For each difference: assessed whether it raises NEW questions of safety or effectiveness
→ If yes → address with performance data. If no → provide written scientific rationale per §807.100(b)(2)(ii)(B).
- ☐ If characteristics are DIFFERENT but no new safety questions arise: data or rationale is documented for each
→ Performance testing, published literature, risk analysis, or standards compliance — one or more must support each claim.
- ☐ If characteristics raise new safety questions: performance data exists and is included in the submission
→ No new safety question can be left unaddressed. If data cannot be obtained, the De Novo pathway may be required.

- ☐ All new patient-contacting materials evaluated per ISO 10993-1:2018 biological evaluation framework
 - New patient-contacting materials require biocompatibility testing, not just literature — risk-based justification must be documented.
- ☐ Software differences classified per FDA guidance (Minor/Moderate/Major) and documented accordingly
 - Moderate and Major software: IEC 62304 lifecycle documentation required. Networked devices: cybersecurity per Sept 2023 FDA guidance.
- ☐ Selected predicate represents the strongest available SE basis — not merely a convenient one
 - Using a technically dissimilar predicate invites additional questions. Choose the most similar legally marketed device available.

PHASE 4 — Strategic Strength Assessment

- ☐ The selected predicate was cleared by the same FDA review division that will review your submission
 - Different review divisions may apply different interpretive standards — confirm the reviewing division via the FDA product code lookup tool.
- ☐ The predicate is sufficiently recent (within 5–7 years) — particularly for software-enabled or materials-advanced devices
 - FDA may question the relevance of predicates from the 1990s for modern software or novel materials — identify a more recent predicate if possible.
- ☐ No more than 1–2 predicates are cited in the primary SE comparison
 - Predicate stacking (5+ predicates) signals a weak single-predicate basis and often triggers FDA additional information requests.
- ☐ The predicate's 510(k) summary or FDA summary has been reviewed for any adverse SE precedent
 - A predicate cleared only after a significant AI request cycle may signal known SE challenges for this device type or indication.
- ☐ A split-predicate strategy (one predicate for intended use, one for technology) has been considered if needed
 - Split predicates are permissible per FDA guidance — each comparison must independently satisfy SE criteria.
- ☐ An independent regulatory affairs review of the predicate rationale has been completed before submission
 - Independent review of predicate logic before submission is the highest-value pre-submission QA activity.

DECISION GATE: If Phase 1 has any unchecked item — stop. The candidate is not a valid 510(k) predicate. If Phases 2–3 have unchecked items — address each before submission or identify a stronger predicate. Only proceed when all applicable items are checked and rationale is documented.

Regulatory basis: 21 CFR §807.100(b); FDA. The 510(k) Program: Evaluating Substantial Equivalence in Premarket Notifications. July 28, 2014.

Template II — Substantial Equivalence Comparison Template

Complete for each predicate cited in the submission. This template maps directly to FDA's substantial equivalence determination criteria under 21 CFR §807.100(b). Retain in the submission Technical File.

SUBMISSION IDENTIFICATION

Device Name:	K-Number (if known):
Applicant / Manufacturer:	Submission Date:
Predicate Device Name:	Predicate K-Number:
Predicate Clearance Date:	Predicate Product Code:
Classification Regulation (21 CFR §):	Device Class:

STEP 1 — Intended Use Comparison [21 CFR §807.100(b)(1)]

ELEMENT	SUBJECT DEVICE	PREDICATE DEVICE	SAME?
Patient Population (age, condition, disease state)			
Clinical Indication (diagnostic, therapeutic,			
Anatomical Site / Body System			
Clinical Setting (hospital, outpatient, home-use)			
Intended User (HCP / lay user / both)			

INTENDED USE CONCLUSION:

If DIFFERENT — written justification:

STEP 2 — Technological Characteristics [21 CFR §807.100(b)(2)]

CHARACTERISTIC	SUBJECT DEVICE	PREDICATE DEVICE	SAME?	NEW SAFETY Q?
Materials (patient-contacting)				
Operating Principle / Energy Type				
Design / Geometry				
Duration of Use				

Software Functionality				
Sterilization Method				
Electrical Characteristics				
Other: _____				

TECH CHARACTERISTICS CONCLUSION:**Evidence basis (testing, rationale, standards):****STEP 3 — Substantial Equivalence Determination****OVERALL SE CONCLUSION:****Prepared by:**

Date:

Reviewed by:

Date:

Regulatory basis: 21 CFR §807.100(b); FDA. The 510(k) Program: Evaluating Substantial Equivalence in Premarket Notifications. July 28, 2014.

Template III — 510(k) Submission Readiness Checklist

Pre-submission verification checklist for Traditional 510(k) submissions. Complete before eCopy submission to minimize FDA Refuse to Accept (RTA) determinations and Additional Information (AI) requests. Based on FDA's Refuse to Accept Policy for 510(k)s (2019) and eCopy Program guidance (2015).

Device Name:

Submission Type:

K-Number (if assigned):

Target Submission Date:

A. Administrative Completeness

- ☐ Cover sheet (FDA Form 3514) completed — all required fields, correct applicant information
→ RTA criterion: incomplete Form 3514 is one of the most common grounds for refuse-to-accept.
- ☐ Indications for Use statement (FDA Form 3881) completed, signed, and consistent with device description
→ The IFU statement on Form 3881 must exactly match the indications language throughout the submission.
- ☐ 510(k) Summary OR 510(k) Statement included — one is required per 21 CFR §807.92
→ 510(k) Summary is more commonly submitted; must include all required elements per §807.92(a)(1)-(7).
- ☐ Financial certification or disclosure statement included per 21 CFR Part 54
→ Required for any submission relying on clinical data covered by the financial disclosure regulation.
- ☐ Truthful and accuracy certification signed by applicant's authorized representative
→ Required under 21 CFR §807.87(k); must be signed by person legally authorized to bind the applicant.
- ☐ User fee payment confirmed — verify current MDUFA fee schedule at [fda.gov](https://www.fda.gov/oc/fees)
→ Submissions without verified user fee payment will not be accepted. Check FDA's current year MDUFA fee schedule.
- ☐ eCopy formatted per FDA guidance — PDF files, correct folder structure, no password protection
→ eCopy Program for Medical Device Submissions guidance (March 2015) specifies exact formatting requirements.

B. Device Description and Predicate / SE Section

- ☐ Device description includes photos, diagrams, dimensions, and component specifications
→ RTA criterion: missing device description is a common grounds for refuse-to-accept.
- ☐ Predicate device(s) identified with full K-number and verified clearance date
→ Every predicate K-number should be verified directly in the FDA 510(k) database before submission.
- ☐ Intended use comparison table included — subject device vs. predicate
→ Required per 21 CFR §807.87(f) and FDA's SE guidance. Must explicitly state SAME or DIFFERENT.
- ☐ Technological characteristics comparison table included with each difference identified
→ Required per 21 CFR §807.87(f). Each technological difference must be accompanied by data or rationale.
- ☐ SE determination explicitly addresses new safety/effectiveness questions for each technological difference
→ FDA requires explicit analysis of each difference per 21 CFR §807.100(b)(2)(ii).
- ☐ If split predicate: separate SE comparison completed for each predicate, each meeting SE criteria independently
→ Split predicates must each independently satisfy intended use and technological characteristics criteria.

C. Performance Testing

- ☐ Bench testing summary included with acceptance criteria and pass/fail results for each test
→ Testing must be conducted on final production-equivalent device or with documented worst-case rationale.
- ☐ All referenced standards cited by full number, title, and edition year
→ Example: IEC 60601-1:2005+AMD1:2012+AMD2:2020 — edition year must match what was actually used.
- ☐ Deviations from applicable standards documented with written justification
→ Do not silently deviate from a standard — document the deviation and provide scientific rationale.
- ☐ Worst-case testing rationale documented for each applicable test
→ FDA expects testing on the worst-case configuration (highest power, largest size, most demanding use case).

- ☐ Animal testing data included with full study design and statistical analysis (if applicable)
→ Required for devices with biological performance endpoints not addressable by bench testing alone.
- ☐ Clinical data included with IRB documentation or exemption rationale (if applicable)
→ If relying on prior clinical data (e.g., PMA supplement), document the evidence bridge explicitly.

D. Biocompatibility — ISO 10993-1:2018

- ☐ Patient contact classification completed — nature (skin, mucosal, blood) and duration (limited, prolonged, permanent)
→ Use ISO 10993-1:2018 Table A.1 for contact classification — this drives the required testing endpoints.
- ☐ Biological evaluation plan (BEP) documented
→ ISO 10993-1:2018 requires a documented BEP as the framework for all biocompatibility decisions.
- ☐ Cytotoxicity, sensitization, and irritation/intracutaneous reactivity addressed at minimum
→ These three endpoints are required for virtually all patient-contacting device categories.
- ☐ Duration-appropriate endpoints addressed: systemic toxicity, genotoxicity, subacute/subchronic/chronic toxicity
→ Required endpoints expand with increasing contact duration — prolonged/permanent contact requires full battery.
- ☐ Implant-specific endpoints addressed: implantation study, hemocompatibility (if blood-contacting)
→ Hemocompatibility testing per ISO 10993-4 is required for blood-contacting devices.
- ☐ Chemical characterization / analytical chemistry data available per ISO 10993-18
→ Risk-based biocompatibility justification must be anchored in chemical characterization data.

E. Sterility and Shelf Life (sterile devices only)

- ☐ Sterility assurance level (SAL) specified — SAL 10⁻⁶ standard for implants and critical devices
→ State the target SAL explicitly and reference the applicable sterilization standard.
- ☐ Sterilization validation completed per applicable standard: AAMI/ISO 11135 (EO), 11137 (radiation), or equivalent
→ Include validation data demonstrating that the sterilization process consistently achieves the stated SAL.
- ☐ Packaging validation completed: seal integrity, distribution simulation, and aging per ASTM F1980/ISTA protocols
→ Accelerated aging data acceptable with real-time aging confirmation commitment.
- ☐ Shelf life claim supported by real-time or accelerated aging data
→ The shelf life stated on labeling must be supported by packaging integrity data across the full shelf life period.

F. Software and Cybersecurity (software-enabled devices)

- ☐ Software level of concern determined: Minor / Moderate / Major — per FDA's 2005 software guidance
→ Level of concern drives the required software documentation: Minor = basic; Moderate/Major = full IEC 62304 lifecycle.
- ☐ IEC 62304 software development lifecycle documentation completed (Moderate and Major only)
→ IEC 62304:2006+AMD1:2015 requires documentation of software development planning, architecture, coding, V&V.
- ☐ Software risk management integrated with ISO 14971:2019 process risk analysis
→ Software hazards must be included in the device-level risk management file.
- ☐ Usability engineering file documented per IEC 62366-1:2015 or ANSI/AAMI HE75
→ FDA increasingly expects usability engineering documentation even for non-software-primary devices.
- ☐ Cybersecurity documentation included per FDA's September 2023 guidance (if networked/connected)
→ Required for any device with external communication interfaces. Must address threat modeling and SBOM.
- ☐ SBOM (Software Bill of Materials) included per FDA Omnibus Reform Act 2022 requirement
→ Machine-readable SBOM required for submissions received on or after October 1, 2023.
- ☐ If AI/ML: Predetermined Change Control Plan (PCCP) included if algorithm modifications anticipated post-clearance
→ PCCP per FDA's April 2023 guidance allows pre-specified algorithm changes without a new 510(k) per modification.

G. Labeling — 21 CFR Part 801

- ☐ Draft labeling (IFU) included and indications match Form 3881 statement exactly
→ Labeling indications must match the IFU on Form 3881 word for word — discrepancies cause RTA.

- ☐ Required 21 CFR Part 801 labeling elements present: device name, manufacturer, directions for use
→ Prescription device statement required per 21 CFR §801.109 if not for OTC/consumer use.
- ☐ All required warnings, precautions, and contraindications included and consistent with testing data
→ Contraindications must be consistent with the patient population exclusions in clinical/bench studies.
- ☐ Symbols used on labeling defined in a symbol glossary per ISO 15223-1
→ FDA requires a symbol glossary for any ISO or IEC symbols used — include as part of the labeling submission.
- ☐ Expiration date / shelf life claim on labeling consistent with packaging and aging test data
→ The shelf life on labeling must not exceed the shelf life supported by packaging integrity testing data.
- ☐ Electronic labeling approach documented if physical IFU is not included in packaging
→ FDA's e-labeling guidance allows electronic IFU delivery under specific conditions — document compliance.

H. Pre-Submission Activities

- ☐ Q-submission (Pre-Sub) meeting held for novel technology, novel predicate strategy, or unclear pathway
→ FDA's Pre-Sub program (Q-submission) is strongly recommended before any submission with novel elements.
- ☐ FDA feedback from Pre-Sub meeting addressed in submission content
→ If FDA provided written guidance in a Pre-Sub response, reference the Q-submission number and address each point.
- ☐ Any applicable FDA guidance documents for this device type identified and compliance addressed
→ Check the FDA Device Advice guidance database for device-type-specific guidance that governs the submission.
- ☐ Quality System compliance (21 CFR Part 820 / ISO 13485:2016) confirmed for design controls
→ Design controls (§820.30) must be in place — FDA may inspect design history file during or after clearance review.

OVERALL SUBMISSION STATUS

READINESS ASSESSMENT:

Outstanding items / notes:

Prepared by:

Date:

Reviewed by / RA Lead:

Date:

RTA RISK: Sections A and B (administrative + SE/predicate) are the primary RTA grounds. Sections C–H drive Additional Information (AI) requests during substantive review. FDA's Refuse to Accept Policy (September 2019) lists all mandatory review criteria.

Sources: FDA. Refuse to Accept Policy for 510(k)s. Sept 13, 2019. FDA. eCopy Program for Medical Device Submissions. March 4, 2015. 21 CFR Part 807 (Subpart E); 21 CFR Part 801; ISO 10993-1:2018; IEC 62304:2006+A1:2015; ISO 14971:2019; FDA. Cybersecurity in Medical Devices: Premarket Submissions. Sept 2023. FDA. Predetermined Change Control Plans for ML-Enabled Devices. April 2023.